OmniTriage AI

TRACK_ID=PS01

Patient Intake Triage Assistant • Gemini 3.5 Flash Lite + Local RAG

Protocols (15)

Judge Quick-Load Scenarios:

Chest Tightness & Dizzy | Severe Dyspnea & Cyanosis | Acute RLQ Stomach Pain | Child 103°F + Stiff Neck | Twisted Ankle (Can Walk)

1 Patient Presentation Intake

Reset

Patient Age: 20

Gender: Female

Known Medical History / Medications

Upload Report (.pdf, .png, .jpg, .txt)

Asthama | Conditions: None reported | Allergies: None reported | Meds: None reported

Extracted from **asthama.jpg**

Chief Complaint (Everyday natural language)

Voice Input | Describe in patient's words

I have a mild chest pain.

Re-Analyze

2 Clinical Follow-Up Clarification

Pending Details

The assistant identified missing clinical parameters (onset duration, pain severity, radiation, or red flags) required to differentiate urgency according to protocol.

1 **How long has this mild chest pain been present, and did it start suddenly?**

Clinical Intent: Establish onset and duration of symptoms

Started suddenly today | Gradual onset over a few days | Present for over a week

Started suddenly today

2 **Does the discomfort spread to your jaw, neck, back, or down either arm?**

Clinical Intent: Assess pain radiation patterns associated with cardiac risks

No radiation | Spreads to my jaw or neck | Spreads down my left arm | Spreads to my back

Spreads to my back

3 **Are you experiencing any other symptoms along with the chest pain, such as shortness of breath, dizziness, or nausea?**

Clinical Intent: Screen for high-risk associated autonomic or respiratory symptoms

None | Shortness of breath | Dizziness or lightheadedness | Nausea or sweating

no

Update Triage Note

Clinical Triage Note

Generated: 2026-09-05 21:54:31 UTC

IMMEDIATE HUMAN CLINICAL ESCALATION REQUIRED RED FLAG

Presence of sudden onset chest pain with radiation to the back matches escalation triggers for potential thoracic catastrophe or acute coronary syndrome.

URGENCY ACUITY

EMERGENCY

RECOMMENDED UNIT

Emergency Department (Resuscitation Bay / Chest Pain Unit)

RECOMMENDED SPECIALIST

Cardiology / Emergency Physician

**Specialist category is for intake routing guidance only and does not constitute a formal medical diagnosis.*

Protocol Guideline Cited: `RULE_CHEST_01` Inspect Protocol >

Suspected Acute Coronary Syndrome (ACS) or Thoracic Catastrophe

Symptoms align with protocol RULE_CHEST_01 indicating potential high-risk thoracic pathology due to sudden onset chest pain radiating to the back.

Patient Reported

- mild chest pain

Follow-Up Findings

- Started suddenly today
- Spreads to my back
- No associated shortness of breath, dizziness, or nausea

Remains Unknown

- Vital signs
- Electrocardiogram (ECG) results
- Cardiac biomarker levels
- Past medical history and cardiovascular risk factors

Clinical Safety Guardrail: TRIAGE DECISION SUPPORT ONLY: This assistant assists patient sorting into clinical acuity streams and does NOT provide a medical diagnosis.

DOCTOR RECOMMENDATION & NEXT STEPS

Awaiting Evaluation

Rest in a comfortable seated position. Do not eat, drink, or exert yourself until evaluated with a 12-lead ECG.

Need this in everyday words?

Explain Report in Plain Language